

YOUR WEBINAR RECAP

EVERYTHING DR. KAHN COVERED ON PROSTATE CANCER & UROLOGIC HEALTH

THANK YOU FOR JOINING US SEPTEMBER 12, 2026

This guide recaps what Dr. Paul Kahn, urologist with over 40 years serving South Florida, actually covered live during our September 12th webinar — real numbers, real screening guidance, and real answers, organized so you can revisit them anytime.

"Early detection is the key." — Dr. Paul Kahn

SECTION 1: THE NUMBERS DR. KAHN SHARED

Prostate cancer is the most common soft-tissue cancer affecting men. Dr. Kahn opened with the statistics that shape everything else in this guide.

- ✓ Roughly 314,000 new prostate cancer cases are diagnosed in the U.S. each year
- ✓ Most men diagnosed do not develop an aggressive form of the disease
- ✓ Still, about 36,000 men die from prostate cancer each year in the U.S.
- ✓ Among men who present with advanced disease (cancer that has spread beyond the prostate), roughly 40% will die within 5 years of diagnosis

Why this matters: prostate cancer often develops without any symptoms — which is exactly why Dr. Kahn emphasized that screening, not waiting for symptoms, is what catches it early.

SECTION 2: WHO IS AT HIGHER RISK

- ✓ Black men are almost twice as likely to develop prostate cancer as men of other groups
- ✓ Men over age 70 are at higher risk
- ✓ A family history of prostate cancer raises your own risk
- ✓ A family history of breast, ovarian, pancreatic, or colon cancer also raises prostate cancer risk — these cancers can share genetic links, including the BRCA1 and BRCA2 genes

Dr. Kahn's take-home message: "You need to discuss with your family about their medical history... we need to know what cancers your parents, your siblings, and your aunts and uncles passed away from." Many families never have this conversation — his advice is to start it.

SECTION 3: THE PSA TEST, EXPLAINED

The Prostate-Specific Antigen (PSA) test measures a protein produced by the prostate. Dr. Kahn was direct about an important point: a high PSA does not automatically mean cancer.

- ✓ BPH (benign prostate enlargement) is the most common cause of an elevated PSA — far more common than cancer
- ✓ Every man's PSA naturally rises a small amount each year as he ages
- ✓ Some infections or inflammation can raise PSA without symptoms
- ✓ Certain aggressive prostate cancers do not produce PSA at all — a normal PSA does not rule out cancer
- ✓ Avoid bicycle riding or sexual activity for 72 hours before a PSA test, as both can temporarily raise the result

Risk Level	Recommended PSA Screening Age
No known risk factors	Around age 45
Black men, or family history of related cancers	Around age 40

SECTION 4: WHAT HAPPENS IF YOUR PSA IS ELEVATED

Dr. Kahn walked through the exact process his office follows when a patient's PSA comes back high — a series of steps designed to avoid unnecessary alarm while still catching real cancer early.

- ✓ Step 1: Repeat the PSA test to rule out a lab error
- ✓ Step 2: Ultrasound the prostate to check for enlargement (BPH), the most common explanation
- ✓ Step 3: If still elevated, a 4K blood test may be ordered — a more specific, genetics-based test that gives a personalized risk percentage. Under 10% risk is typically monitored over time; higher results lead to further evaluation
- ✓ Step 4: A digital rectal exam and, if needed, an MRI — which is roughly 80% accurate and can miss about 20% of prostate cancers, especially less aggressive ones ("MRI-invisible" cancers)
- ✓ Step 5: If something suspicious is found, a targeted needle biopsy may be recommended

About the biopsy: Dr. Kahn described the modern approach — a transperineal fusion biopsy, performed with a needle (no cutting), guided in real time by the earlier MRI images. The procedure takes about 10 minutes under anesthesia, patients go home the same day with minimal pain, and results typically take 10 to 14 days.

SECTION 5: TREATMENT OPTIONS IF CAUGHT EARLY

"If your cancer is found early, meaning that it's still contained within the prostate... it is very easy to treat," Dr. Kahn explained. Options at that stage include:

- ✓ Prostatectomy — surgical removal of the prostate
- ✓ Radiation therapy directed at the prostate
- ✓ Focal treatment — treating only the specific area of cancer rather than the whole prostate, including cryoablation (freezing the tumor) or HIFU (focused ultrasound)

Worth knowing: after prostate removal, most men experience some erectile dysfunction, which Dr. Kahn noted is typically long-term, while bladder control issues are usually temporary and resolve within weeks. He emphasized that his practice discusses these outcomes openly with patients and their partners before surgery, addressing both the physical and emotional impact.

SECTION 6: BUSTING A COMMON MYTH ABOUT BIOPSIES

During the live Q&A, a question came up that Dr. Kahn said he hears constantly: does a biopsy needle cause cancer to spread? This belief keeps some people from getting screened at all.

Dr. Kahn's answer: this is a myth, not a risk. "A cancer cell is a living cell — it needs sugar, it needs oxygen, it needs blood." A biopsy sample cannot survive and spread once removed from its blood supply.

If concern about this myth has ever delayed a biopsy for you or someone you know, this is worth sharing directly — it may be the one thing standing between someone and an early diagnosis.

SECTION 7: GENETIC TESTING — A NEWER PIECE OF THE PUZZLE

- ✓ After a biopsy, a test called the Decipher test can examine gene mutations within the cancer cells themselves
- ✓ This helps predict how likely the cancer is to behave aggressively over time
- ✓ It is used alongside — not instead of — the traditional Gleason score, which pathologists use to grade how cancer cells look under a microscope
- ✓ Genetic testing has expanded beyond the BRCA1 and BRCA2 genes discussed in Section 2 — more genetic markers are now part of routine risk assessment
- ✓ Age and life expectancy factor into what results mean in practice — a younger patient with even a low-risk result is typically still treated, given a longer expected lifespan ahead

SECTION 8: SCREENING, AGE, AND YOUR OWN DOCTOR'S JUDGMENT

Dr. Kahn shared a personal story about a patient whose PSA testing was stopped in his 90s, given his age. Years later, that patient was diagnosed with cancer that had already spread widely, and passed away shortly after. Dr. Kahn now discusses PSA testing with patients "regardless of age."

An important note: this reflects Dr. Kahn's own clinical experience and philosophy, not a single universal rule for every man at every age. Broader medical guidance generally recommends an individualized conversation between you and your own doctor about the benefits and limits of continued screening as you age, taking your overall health and life expectancy into account. The real takeaway: age alone should never be the only reason screening stops — that decision belongs in a conversation with your doctor, not an assumption.

SECTION 9: HOW UROLOGIC HEALTH CONNECTS TO YOUR WHOLE BODY

Dr. James Gavin, CHP's chronic disease specialist, closed the medical discussion by tying urologic health back to the same "connected chronic disease" theme running through this year's CHP webinar series.

- ✓ Chronic inflammation is a common thread across many conditions — including narrowed, clogged, or inflamed blood vessels that can affect urologic health

- ✓ High blood pressure, diabetes, and dyslipidemia (unhealthy cholesterol) are shared risk factors that show up again and again across chronic conditions
- ✓ Erectile dysfunction is common — and can be an early warning sign of underlying cardiac risk. Dr. Kahn noted that all men with erectile dysfunction should be evaluated by cardiology
- ✓ For men, urologists typically manage both urinary and reproductive health together. For women, this is usually split between a urologist and a gynecologist

Dr. Gavin's framing: *"It is rare for a person to come in, and all they have in this array of conditions is one thing wrong." A problem in one system is worth treating as a signal to look at the whole picture, not just the one symptom in front of you.*

SECTION 10: PROSTATITIS AND URINARY INCONTINENCE

Two related conditions came up during the webinar that are often confused with prostate cancer itself, but are distinct and generally more common.

- ✓ Prostatitis is inflammation or infection of the prostate gland (similar to cystitis in women). It can be acute (a first, initial infection) or chronic (recurring episodes), and either bacterial or non-bacterial in nature
- ✓ Urinary incontinence affects both men and women. In women, stress incontinence (leaking during coughing or sneezing) is common after menopause, while urge incontinence (not making it to the bathroom in time) can affect both sexes
- ✓ Fewer than 1% of men experience incontinence severe enough to require surgery after prostate surgery
- ✓ Treatment for incontinence typically starts conservative, moving to medication or surgery depending on the specific type

SECTION 11: URINARY TRACT INFECTIONS IN WOMEN

Dr. Kahn noted that on any given day, roughly a third of the patients in his practice are dealing with a urinary tract infection (UTI) — most commonly linked to sexual activity, when bacteria are pushed into the urethra and bladder.

- ✓ Common symptoms include frequency, burning during urination, urgency, and leakage
- ✓ Diagnosis involves examining urine under a microscope and sending a culture to confirm the correct antibiotic
- ✓ Women with recurrent infections after intercourse may be treated preventively, sometimes including topical (not oral) estrogen — which Dr. Kahn described as very safe when applied as a cream or gel rather than taken as a pill
- ✓ Simple UTIs are easy to manage; infections that reach the kidney are more serious and may require hospitalization and IV antibiotics
- ✓ Prevention includes staying well hydrated and emptying the bladder before and after intercourse

Worth knowing: Dr. Kahn cautioned against antibiotics being prescribed without a proper urine culture first — a common shortcut at some urgent care visits that can lead to using the wrong medication or missing the actual cause of symptoms.

SECTION 12: DR. KAHN'S TAKE-HOME MESSAGES

1

Know your family history

Talk to parents, siblings, aunts, and uncles about what cancers run in your family — not just prostate cancer, but breast, ovarian, pancreatic, and colon cancer too.

2

Do not wait for symptoms

By the time symptoms appear, prostate cancer has often already spread beyond the prostate. Screening catches it earlier than symptoms ever will.

3

Know your PSA and track it over time

A single number matters less than how it changes year to year.

4

Do not panic over an elevated PSA

BPH, not cancer, is the most common cause. A clear process exists to figure out what is actually going on.

5

Have the conversation with your doctor

Early detection remains the single biggest factor in successful treatment.

WHAT YOU JUST GOT, FOR FREE

A full hour with a urologist of over 40 years, a chronic disease specialist, and an emergency physician — the kind of combined expertise that would typically mean three separate appointments. You now have real screening ages, a clear step-by-step diagnostic pathway, genuine treatment options, and a myth cleared up that keeps some people from getting screened at all.

ONE THING TO DO RIGHT NOW

Send this guide to one person in your life who has been putting off this conversation. That single share may matter more than anything else in this document.

WHAT'S NEXT

On October 10th, Dr. Steven Hahn joins CHP for "From Mouth to Gut — And Beyond," part one of a two-part series on digestive health and how it connects to the rest of your body. Free, live, same format you just experienced.

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This guide summarizes a live discussion for community education purposes and does not replace individualized medical advice. Always consult your own doctor or urologist about your specific health.